



**CHRISTIAN MEDICAL COLLEGE
VELLORE - 4
DEPARTMENT OF HAEMATOLOGY
BONE MARROW TRANSPLANT**

**DEATH SUMMARY
PRELIMINARY REPORT**

Name : TARIQ AL FAYAD M

Age : 22

Sex : Male

Ward : BMTU

Hospital Number : 784318H

MRDNo : 551807J

Admitted On : 23-Feb-2020

Expired On :

Address : AMMAPATTINAM

958 SOUTH 4TH STREET

MANAMELKUDI PUDUKKOTTAMIL NADU

Pincode : 614617

Cause of Death: **NON SEVERE APLASIC ANAEMIA**

PNH CLONE-NOT DETECTED

STRESS CYTOGENETICS NOT SENSITIVE - MITOMYCIN C

History

Mr Tariq Al Fayad M, 22 year old boy, optometrist working in the Shankar eye hospital from Pudukkottai TN currently settled in chennai admitted for matched sibling donor allogenic transplant from younger brother (HLA-10/10).

He is asymptomatic at the time of admission.

HISTORY OF PRESENTING ILLNESS

Mr Tariq had complained of sudden onset floaters (blackish spots) for 1 day in Sep 2019, He was evaluated in Sankar Eye Hospital and found to have superficial haemorrhage on the left optic disc. He was further evaluated under the department of Medicine and was found to have pancytopenia with Hb-6.3gm%; Platelet-90,000 and TLC-4120 with N47&L45. For further work up bone marrow examination was done and it was hypocellular marrow and diagnosed as aplastic anaemia. He came to CMC for further management. At CMC peripheral blood showed pancytopenia and repeat bone marrow done which suggestive of Aplastic anaemia with cytogenetics was Normal Karyotype.

In view of peripheral platelet count > 20,000 and ANC > 500, final impression was Non Severe Aplastic Anemia.

Stress cytogenetics was done to rule out Bone Marrow Failure syndrome which was not sensitive to Mitomycin -C.

Patient counselled regarding diseases and treatment options. In view of financial constraints patient was started with Tab Stanazolol 10 mg twice daily since december 2019. However there was no response and patient need blood products supports at regular interval, till now he received 8 packed cell and 8 platelet transfusions.

Now, Mr Tariq was admitted with allogenic matched sibling transplant from younger brother.

Past History

Non significant

Family history

Parents had 2 degree consanguineous marriage. He had only one siblings, younger



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brother.

Personal history

Unmarried, mixed diet with normal bowel and bladder habits.

Funds

Self - 12 lacs

Via Insurance - Star Health and allied insurance limited.

On Examination

There was no pallor, cyanosis, or edema. Clubbing is present.
Lymph nodes were not palpable

Blood Pressure: 110/80 mmhg, Pulse: 80 beats/min, Respiratory rate: 24 breaths/min,
Temperature: 100.4 Degree F

Systemic Examination:

Abdomen: Soft, nontender, liver and spleen were not palpable. There was no free fluid or other mass palpable

Cardiovascular: Both heart sound heard normally

Respiratory: Air entry was normal and equal on both sides of chest

Nervous system: was normal

Rest of systemic examination was normal

Investigations

23/02/2020	HAEMOGLOBIN	7.2	GM%	
17/02/2020	HAEMOGLOBIN	6.9	GM%	
05/02/2020	HAEMOGLOBIN	8.3	GM%	
03/02/2020	HAEMOGLOBIN	8.2	GM%	
31/01/2020	HAEMOGLOBIN	8.2	GM%	
29/01/2020	HAEMOGLOBIN	5.3	GM%	
16/01/2020	HAEMOGLOBIN	5.9	GM%	
Aniso+, tear drop cells, polychromasia				
prt., Ms. Jinu Sebastian/Dr Abha				
10/02/2020	RETICULOCYTE COUNT	2.21	%	0.5 - 2.5
23/02/2020	PLATELET COUNT	24000	/ cumm	
1,50,000-4,50,000				
17/02/2020	PLATELET COUNT	19000	/ cumm	
1,50,000-4,50,000				
05/02/2020	PLATELET COUNT	8000	/ cumm	
1,50,000-4,50,000				
03/02/2020	PLATELET COUNT	4000	/ cumm	
1,50,000-4,50,000				
Plt count on smear < 15,000/cumm. Mr. Sam				
31/01/2020	PLATELET COUNT	19000	/ cumm	
1,50,000-4,50,000				
29/01/2020	PLATELET COUNT	12000	/ cumm	
1,50,000-4,50,000				
16/01/2020	PLATELET COUNT	27000	/ cumm	
1,50,000-4,50,000				
23/02/2020	WBC TOTAL	3500	/ cumm	
4,000-12,000				
17/02/2020	WBC TOTAL	4000	/ cumm	
4,000-12,000				
05/02/2020	WBC TOTAL	3600	/ cumm	
4,000-12,000				
03/02/2020	WBC TOTAL	3600	/ cumm	
4,000-12,000				
31/01/2020	WBC TOTAL	1800	/ cumm	
4,000-12,000				
29/01/2020	WBC TOTAL	1100	/ cumm	
4,000-12,000				
16/01/2020	WBC TOTAL	5100	/ cumm	
4,000-12,000				

23/02/2020	WBC DIFFERENTIAL		
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	31	%
	BASOPHILS	1	%
	LYMPHOCYTES	0	%
	MONOCYTES	60	%
	NUCL RED CELLS	8	%
17/02/2020	WBC DIFFERENTIAL	0.2	/100 WBC
	MRS.S.Latha/dr.krishna		
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	34	%
	BASOPHILS	1	%
	LYMPHOCYTES	0	%
	MONOCYTES	59	%
	NUCL RED CELLS	6	%
05/02/2020	WBC DIFFERENTIAL	0.49	/100 WBC
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	34	%
	BASOPHILS	1	%
	LYMPHOCYTES	0	%
	MONOCYTES	60	%
	NUCL RED CELLS	5	%
03/02/2020	WBC DIFFERENTIAL	1.17	/100 WBC
	Mr.Sam/Dr Don		
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	18	%
	BASOPHILS	1	%
	LYMPHOCYTES	0	%
	MONOCYTES	73	%
	NUCL RED CELLS	8	%
31/01/2020	WBC DIFFERENTIAL	1.7	/100 WBC
	Mr.santhosh/Dr.Gokul.G		
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	3	%
	BASOPHILS	0	%
	LYMPHOCYTES	0	%
	MONOCYTES	21	%
	NUCL RED CELLS	1	%
29/01/2020	WBC DIFFERENTIAL	1.7	/100 WBC
	DC done 25 cells. Mr.Sam/Dr Abha		
	BLASTS		%
	PROMYELOCYTES		%
	MYELOCYTES		%
	METAMYELOCYTES		%
	BANDFORMS		%
	NEUTROPHILS		%
	EOSINOPHILS	9	%
		0	%

16/01/2020	BASOPHILS	0	%	
	LYMPHOCYTES	14	%	
	MONOCYTES	2	%	
	NUCL RED CELLS	0.9	/100 WBC	
	WBC DIFFERENTIAL			
	BLASTS		%	
	PROMYELOCYTES		%	
	MYELOCYTES		%	
	METAMYELOCYTES		%	
	BANDFORMS		%	
24/02/2020	NEUTROPHILS	33	%	
	EOSINOPHILS	1	%	
	BASOPHILS	0	%	
	LYMPHOCYTES	60	%	
	MONOCYTES	6	%	
	NUCL RED CELLS	0.57	/100 WBC	
	MANUAL URINE MICROSCOPY & CRYSTALS			
	RBC:6 /hpf, WBC:0-1 /hpf, EC:0-1 /hpf,			
	BACT:NIL, YEAST CELLS:NIL, MUCUS:NIL			
	WBC			
05/02/2020	RBC		MM/cumm	M 4.4-5.9;F
	3.8-5.2			
	EPITHELEAL CELL			
	CASTS			
	CRYSTALS			
	PT WITH INR			
	PATIENT	13.3	SECS	
	NORMAL RANGE	11.7-16.1	SECS	
	INR	0.95		
	1/2Pt+1/2 Cont		SECS	
13/02/2020	BONE MARROW			
	Impression: Dilute marrow, inadequate for			
	evaluation By DR GOKUL G./DR ARPANA PALLE			
	CBC PROFILE			
	NE:40, LY:51, MO:8, EO:1, BA:0			
	MCV	87.5	fL	80-100
	MCH	30.4	pG	26-34
	MCHC	34.7	%	31.4-36.3
	RDW	16.0	%	11.5-14.5
	TOTAL WBC	4200	/CUMM	
10/02/2020	4,000-12,000			
	HB	6.9	g/dL	M 13-17;F
	11-15			
	PLATELET COUNT	14000	/CUMM	
	1,50,000-4,50,000			
	RBC	2.28	MM/cumm	M 4.4-5.9;F
	3.8-5.2			
	HCT (PCV)	19.9	%	M 40-50;F
	35-46			
	RETICS		%	0.5-2.5
05/02/2020	APTT			
	PATIENT	32.0	SECS	
	NORMAL RANGE	27.8-40.4		
	1/2Pt+1/2 Cont		SECS	
	24/02/2020			
	URINALYSIS ROUTINE	Auto Validated		
	RBC:6 /hpf, WBC:0-1 /hpf, EC:0-1 /hpf,			
	BACT:NIL, YEAST CELLS:NIL, MUCUS:NIL			
	GLUCOSE	Negative		
	BILIRUBIN	Negative		
1.025	KETONE	Negative		
	SPECI.GRAVITY	1.011		1.015 -
	BLOOD	Negative		
	PH	5.5		4.6 - 8.0
	PROTEIN	Negative		
	UROBILINOGEN	normal		
	NITRITE	Negative		
	LEUKOCYTES	Negative		
	RBC		MM/cumm	M 4.4-5.9;F
	3.8-5.2			
29/01/2020	WBC			
	MANUAL URINE MICROSCOPY & ROUTINE URINALYSIS	Auto Validated		

	BACT:NIL, YEAST CELLS:NIL, MUCUS:NIL			
	GLUCOSE	Negative		
	BILIRUBIN	Negative		
	KETONE	Negative		
1.025	SPECI.GRAVITY	1.024		1.015 -
	BLOOD			
	PH	Negative		
	PROTEIN	7.0		4.6 - 8.0
	UROBILINOGEN	+		
	NITRITE	+		
	LEUKOCYTES	Negative		
	RBC	Negative		
3.8-5.2		8 /hpf	MM/cumm	M 4.4-5.9;F
	WBC	0-1 /hpf		
	EPITHELEAL CELL	1-2 /hpf		
	CASTS			
	CRYSTALS			
05/02/2020	BLOOD GROUPING & RH TYPING	NIL		
24/02/2020	Antibody Screen Negative 45330.62.2	A Rh Positive		
	LFT			
	BILIRUBIN TOTAL	0.60	mg/dL	0.5-1.0
	DIRECT	0.26	mg%	
	PROTEIN TOTAL	8.0	g/dL	6.0-8.5
	ALBUMIN	3.9	g/dL	3.5-5.0
	AST (SGOT)	13	U/L	8-40
	ALT (SGPT)	20	U/L	5-35
	ALKALINE PHOSPHATASE	80	U/L	40-125
05/02/2020	LFT			
	BILIRUBIN TOTAL	0.72	mg/dL	0.5-1.0
	DIRECT	0.39	mg%	
	PROTEIN TOTAL	8.7	g/dL	6.0-8.5
	ALBUMIN	4.1	g/dL	3.5-5.0
	AST (SGOT)	33	U/L	8-40
	ALT (SGPT)	41	U/L	5-35
	ALKALINE PHOSPHATASE	75	U/L	40-125
29/01/2020	LFT			
	BILIRUBIN TOTAL	0.59	mg/dL	0.5-1.0
	DIRECT	0.37	mg%	
	PROTEIN TOTAL	7.4	g/dL	6.0-8.5
	ALBUMIN	3.9	g/dL	3.5-5.0
	AST (SGOT)	60	U/L	8-40
	ALT (SGPT)	48	U/L	5-35
	ALKALINE PHOSPHATASE	70	U/L	40-125
24/02/2020	CREATININE	0.53	mg%	0.5-1.4
23/02/2020	CREATININE	0.65	mg%	0.5-1.4
05/02/2020	CREATININE	0.71	mg%	0.5-1.4
03/02/2020	CREATININE	0.55	mg%	0.5-1.4
31/01/2020	CREATININE	0.67	mg%	0.5-1.4
29/01/2020	CREATININE	0.76	mg%	0.5-1.4
05/02/2020	GLUCOSE RAN PL	124	mg/dL	70-140
23/02/2020	PROCALCITONIN	0.07	ng/ml	
30/01/2020	PROCALCITONIN	0.11	ng/ml	
24/02/2020	Na/K STAT (HEPARIN)			
	SODIUM	137	m mol/L	135-145
	POTASSIUM STAT (HEPARIN)	4.1	m mol/L	3.2-4.7
31/01/2020	Na/K STAT (HEPARIN)			
	SODIUM	139	m mol/L	135-145
	POTASSIUM STAT (HEPARIN)	3.9	m mol/L	3.2-4.7
23/02/2020	CULTURE BLOOD			
05/02/2020	C/S THROAT COMMON			
	Normal Flora			
29/01/2020	BLOOD CULTURE AND SUSCEPTIBILITY BY VITEK 2			
	Final Report-No Growth			
05/02/2020	ECG(CARDIOLOGY)	REPORT UPLOADED		
05/02/2020	ECHO - ADULT - CARDIOLOGY	REPORT UPLOADED		
05/02/2020	CMV TITRE [POSITIVE]	79.4 U/mL		
05/02/2020	ANTI HBS	69	mIU/mL	
05/02/2020	CMV-IGM	NEGATIVE		
	CMV-IgM-INTERPRET THE RESULT WITH CAUTION			
	QUANTITATIVE VIRAL DNA ESTIMATION IS			
	REQUIRED ACTIVE TO CONFIRM CMV DISEASE.			

05/02/2020	BLOOD BORNE VIRUS SCREEN	
	HIV	NEGATIVE
	HbsAg	NEGATIVE
	HCV	NEGATIVE
13/02/2020	CMC - SPECIAL BIOPSIES	6285/20
	Aplastic marrow, trephine biopsy.	
	Reported by : Dr. Immanuel Paul Thayakaran D.	
	Reported on : 17/02/2020 00:35:15	
13/02/2020	SPIROMETRY AND FLOW VOLUME LOOP	REPORT UPLOADED
05/02/2020	LUNG VOLUMES	
05/02/2020	DLCO	
13/02/2020	RMU - SEMEN ANALYSIS	
	e	

Discussion

Mr Tariq Al Fayad M, 22 year old boy from chennai admitted for matched sibling donor allogeneic transplant from younger brother (HLA-10/10).

Patient was started with Conditioning regimen Fludarabine and Cyclophosphamide from 28th Feb 2020 to 4th March 2020. He transfused stem cell with dose 10×10^6 cells/kg with volume-130 ml. Mr Tariq received GVHD prophylaxis cyclophosphamide for 2 days.

Patient had fever with chills on Day +5, blood culture grows Ecoli which was ESBL, procalcitonin was low, he was started with meropenem and colistin. ASs fever spikes were continue empirically Teicoplanin was added and Growth factor was also started. However his fever spikes were continued and patient had hypotensive episodes on Day+10, Infusion Noradrenaline started and Chest-HRCT done which suggestive of Multiple Consolidations in B/L lungs. Patient's galactomanan was sent and upgraded antifungal coverage to Inj Caspofungin and shifted to AICU.

In AICU, he developed acute onset breathlessness with tachycardia and hypotension. Arterial blood gas showed hypoxemia and metabolic acidosis with lactates of 12.5 mmol/lit. It was a warm shock. Again Blood cultures drawn and he was upgarded antibiotics coverage to Inj Tigecycline; Inj Ceftazidime Avibactam. Blood culture grew again GNB. He was started on positive pressure ventilation. In view of neutropenia and documented bacterial infection, he was supported with buffy coat infusions. His work of breathing increased, hence was intubated for ventilation. Blood investigations at that point of time were in favour of septic shock. Despite maximal treatment, he continued to deteriorate and succumbed to his illness on 18/03/2020. Patient died with immediated effect of Septic Shock at 9:00 AM on 18/03/2020.

THIS SUMMARY HAS NOT BEEN FINALISED. PLEASE ENSURE THAT YOU COLLECT YOUR FINAL REPORT

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